

Budget Narrative - Overall Expenditures

The following table summarizes the State's SF-424A overall expenditures for each of the five performance years:

TABLE 1 Overall Expenditures					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Salary	\$255,517.37	\$534,534	\$561,261	\$589,324	\$618,790
B. Fringe	\$97,520.91	\$259,360	\$264,957	\$270,833	\$277,004
C. Travel	\$49,949.50	\$38,393	\$38,393	\$38,393	\$38,393
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$18,212.15	\$20,846	\$20,846	\$20,846	\$20,846
F. Contract	\$221,397,605.96	\$99,132,880	\$103,509,403	\$96,466,278	\$113,390,150
H. Other	\$28,678.16	\$119,911,113	\$115,381,939	\$122,255,739	\$105,155,739
I. Direct Charges	\$221,847,484.05	\$219,897,126	\$219,399,667	\$219,641,413	\$219,500,922
J. Indirect	\$50,523.77	\$2,000,881	\$2,121,208	\$2,256,594	\$2,397,085
Total	\$221,898,007.82	\$221,898,007.82	\$221,898,007.82	\$221,898,007.82	\$221,898,007.82

Budget Narrative – Administrative Expenditures

Table 2 summarizes the SF424A expenditure plan for Administrative Expenditures for each of the five performance years.

TABLE 2 Administrative Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel	\$96,518.75	\$508,980	\$508,980	\$508,980	\$508,980
B. Fringe	\$35,954.09	\$254,010	\$254,010	\$254,010	\$254,010
C. Travel	\$28,729.50	\$30,142	\$30,142	\$30,142	\$30,142
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$5,639.25	\$27,602	\$27,602	\$27,602	\$27,602
F. Contract	\$8,561,279.23	\$6,653,474	\$6,653,474	\$6,653,474	\$6,653,474
H. Other	\$0	\$0	\$0	\$0	\$0
I. Direct Charges	\$8,728,120.82	\$7,474,207	\$7,474,207	\$7,474,207	\$7,474,207
J. Indirect Charges	\$0	\$0	\$0	\$0	\$0
Total	\$8,728,120.82	\$7,474,207	\$7,474,207	\$7,474,207	\$7,474,207

For the ten-month Budget Period 1 (December 31, 2025, to October 30, 2026), the State has budgeted the following amounts by category:

TABLE 2.1 Year 1 Personnel – Salary and Wage					
Salary and Wages	Annual Salary	Total FTE	In-Kind	Months	Total Year 1 Cost
Chief of Staff/PI, Matthew Lara	\$135,300.00	0.4	0	10	\$45,100.00
Secretary/AOR, Janet Stanek	\$215,250.00	0	0.4	10	\$0
Chief Medical Officer, Dereck Totten, MD	\$134,524.00	0	0.15	10	\$0
Deputy Chief of Staff, Lauren Evans	\$89,385.00	0.6	0	5	\$22,346.25
RHTP Program Manager, Vacant	\$58,924.00	1	0	4	\$19,641.33
Fiscal Analyst, Michaela Kesler	\$79,463.00	0.1	0	10	\$6,621.92
Contract Specialist, Samantha Ragle	\$44,948.00	0.15	0	5	\$2,809.25
TOTALS		2.25	0.55		\$96,518.75

The Kansas Department of Health and Environment (KDHE) Chief of Staff will serve as Project Director, devoting 0.40 FTE to providing oversight, facilitating partnerships, setting goals, and monitoring progress. The Project Director is committed to maintaining project staff and leveraging resources across the agency. KDHE's **Secretary** will serve as the Authorized Organizational Representative, devoting 0.40 FTE (in-kind). KDHE's **Chief Medical Officer** will devote 0.15 FTE (in-kind).

The **Deputy Chief of Staff (0.6 FTE)** and **RHTP Program Manager (1.0 FTE)** will support overall coordination, day-to-day program management, and facilitate coordination with internal and external partners. These roles will work with the Project Director to negotiate contracts and sub-recipient agreements and will work directly with vendors to ensure deliverables are met and invoices/expenditure reports are submitted appropriately and timely. The Project Director and Program Manager will be further supported by the Fiscal Analyst (0.1 FTE) and Contract Specialist (1.0 FTE).

The State also has budgeted for fringe benefits for the personnel identified above:

TABLE 2.2 Year 1 Personnel – Fringe Benefits			
Fringe Benefit	Rate	Salary Requested	Total Year 1 Cost
FICA	.07650	\$96,518.75	
UIA	.00000		
SLPA	.00470		
Worker's Compensation	.00140		
Retirement (KPERs)	.12680		
Fringe Total	20.94%		\$20,211.03
Insurance	\$14,817 x 1.06 FTE (accounts for months in role)		\$15,743.06
Fringe/Ins Benefit Total			\$35,954.09

Travel costs are calculated using current State of Kansas and GSA rates. In-state travel is intended for senior staff to conduct RHT Program-related on-site meetings. Out-of-state travel is requested for CMS-hosted grantee meetings and/or national conferences for 1 staff member. All destinations are estimated to be in High-Cost Areas.

TABLE 2.3 Year 1 Program Management Personnel - In-State Travel					
Unit	Quantity of units	Cost Per Unit	Number of Staff	Number of Trips	Total Year 1 Cost
Staff miles (average miles per trip per staff)	180	\$0.725	9	15	\$17,617.50
Vehicle Rental	1	\$40.00	5	15	\$3,000.00
Hotel	1	\$110.00	5	6	\$3,300.00
Per Diem	1	\$68.00	5	10	\$3,400.00
Total					\$27,317.50

TABLE 2.4 Program Management Personnel - CMS Conference Travel		
Item	Rate	Total Year 1 Cost
Airfare	\$700 round trip x 1 staff	\$700.00
Per Diem	\$86/day x 2 days x 1 staff	\$172.00
Lodging	\$350/night x 1 night x 1 staff	\$350.00
Parking	\$25/day x 2 days x 1 staff	\$50.00
Ground Transport	\$50/day x 2 days x 1 staff	\$100.00
K-Tag (tolls)	\$10/trip x 1 staff	\$10.00
Baggage	\$30/trip x 1 staff	\$30.00
Total		\$1,412.00

All supplies will be used for staff previously identified solely to perform RHT Program-related activities. Pursuant to established policy, all KDHE computers are leased on a four-year cycle; amounts requested are the per-year costs for the lease of standard-issue laptops, monitors, keyboards, and docking stations. Office supplies are standard items, e.g., pens, paper, printer ink, with the amount based on State average spend per FTE for these supplies.

TABLE 2.5 Personnel - Supplies			
Item	Rate	Units	Total Year 1 Cost
Laptops	\$679.00	2.25	\$1,527.75
Adobe Acrobat Pro	\$235.00	2.25	\$528.75
Zoom	\$160.00	2.25	\$360.00
Grammarly	\$266.00	2.25	\$598.50
Zscaler + Duo	\$180.00	2.25	\$405.00
Cell Phone	\$554.00	1	\$554.00
Mobile Tablet	\$360.00	1	\$360.00
Monitors	\$268.00	1	\$268.00
Docking Station	\$266.00	1	\$266.00
Computer Supplies (charging cords, ethernet cables, etc.)	\$200.00	2	\$400.00
Office Supplies	\$165.00	2.25	\$371.25
Total			\$5,639.25

The Year 1 administrative expenditures listed under “Contract” include a subrecipient agreement KDHE will enter with its sister agency, the Kansas Department of Aging and Disability Services. This subrecipient agreement includes the following central costs:

TABLE 3.1 Expenses Included in KDADS Subrecipient Agreement				
Personnel and Fringe				
Position	Annual Salary and Benefits	FTEs	Months	Total Year 1 Cost
KDADS RHTP Program Manager	\$74,202.00	1	5	\$31,164.84
KDADS Quality Assurance Specialist	\$90,405.00	2	5	\$75,940.20
Travel				
Purpose	Miles @ .725/mile			Total Year 1 Cost
KDADS RHTP Related Travel	5,700			\$4,132.50
Supplies				
Description	Price Per Unit	Number		Total Year 1 Cost
KDADS Supplies	\$2,984.00	3		\$8,952.00
Other				
Description	Total Year 1 Cost			

KDADS Other Expenses	\$17,487.00
Indirect	
Description	Total Year 1 Cost
KDADS (15.6%)	\$21,477.54

The Kansas Department for Aging and Disability Services (KDADS) will hire a Program Manager (with annual salary and benefits of \$74,202 for five months in Year 1) and two Quality Assurance Specialists (with annual salary and benefits of \$90,405 for five months in Year 1) to facilitate their projects. The State has budgeted \$4,133 for travel expenses for these three individuals, \$17,487 for other related expenses, and \$2,984 per individual for supplies. These supplies include the same items at the same costs as those listed in Table 2.5 for KDHE staff.

The Year 1 administrative expenditures listed under “Contract” also include the following contracts into which KDHE has or will enter:

TABLE 4 Administrative Contracts		
Contracting Party	Administrative Services	Year 1 Cost
Boston Consulting Group	Consulting Services	\$4,750,000.00
BDO Government Services	Fiscal Agent	\$2,329,929.00
Fort Hays State University Docking Institute of Public Affairs	Evaluation Surveys	\$237,858.00
UKHS Care Collaborative Association	Grant assistance	\$1,085,320.80
Total		\$8,403,107.80

Boston Consulting Group (BCG). KDHE will contract with BCG to provide rapid response implementation support for the Kansas RHTP, which will support in-depth state-level infrastructure development and improvement. This work will support staffing and process infrastructure at KDHE to ensure the successful implementation and management of Kansas RHTP initiatives throughout the award period. This will include infrastructure to support RHTP-specific fiscal processes, ensuring satisfactory tracking and compliance with federal award terms.

BDO Government Services (BDO GS). KDHE will contract with a fiscal agent for the RHT Program, BDO Government Services, to provide the financial stewardship, compliance discipline, and audit-ready documentation needed to protect program funds and support the successful delivery of outcomes. Their leaders specialize in supporting state agencies across the full life cycle of federally funded programs, including first-time funding programs that require strong controls and clear processes from the start. They bring nationally recognized grant management capability,

with leadership experience managing programs valued at more than \$127 billion across 18 states and territories, grounded in continuous process improvement, compliance, and efficient execution.

BDO GS will implement a structured, compliance-driven fiscal management approach aligned to Uniform Guidance expectations and designed to keep RHT Program funds properly managed and well documented. This includes establishing a clear financial management framework and controls that support compliant financial management, effective monitoring of expenditures, and timely, accurate reporting to optimize cash flow. They will support award acceptance and grant agreement execution by helping ensure grant requirements are clearly understood, compliance and financial risks are mitigated early, and expectations and responsibilities with subrecipients and implementing partners are clearly established from a fiscal and documentation perspective.

Throughout implementation, BDO GS will support oversight through consistent financial monitoring and documentation practices that keep files organized and ready for review. BDO GS has performed more than 90,000 closeout file reviews and monitoring visits, demonstrating our ability to maintain organized, compliant records suitable for straightforward oversight agency review and approval. They will also implement a proven process to transfer knowledge for a seamless transition at closeout, so KDHE retains the financial records, context, and documentation needed to sustain program integrity through final reporting and final resolution of obligations.

Audit support is a core part of the fiscal agent role. BDO was founded as a CPA firm, and the team has participated in audits by multiple federal and state agencies. BDO will support KDHE staff during audits through efficient communication and complete documentation and will work collaboratively to address any identified areas for improvement.

KHI / Fort Hays State University Docking Institute of Public Affairs. KDHE will contract KHI, in partnership with the Docking Institute, to design, administer, and analyze the results of regional and statewide rural healthcare consumer, provider, and CEO surveys regarding access and experience as a means to evaluate the impact of the State's RHT Plan. The State selected these partners for this work based on their significant experience with Kansas-specific consumer surveys. The initial survey will be performed in Year 1 with two follow-up surveys in Year 3 and Year 5. The cost of the surveys will be \$237,858.

UKHS Care Collaborative Association (CCA). KDHE will contract with CCA for grant assistance services for all programs, although its level of engagement for each program will vary depending on the role assumed by the State and the role assigned to third parties.

CCA, which is a wholly owned subsidiary of the University of Kansas Hospital Authority (d/b/a

The University of Kansas Health System (TUKHS)), was selected to provide services based on proven leadership in rural health initiatives, strong working relationships with rural providers statewide, and engagement in Plan development. The CCA team includes two full-time executives. Dr. Robert Moser, MD, a former Secretary of KDHE and inaugural Executive Director for the Center for Rural Health at the University of Kansas Medical Center, served as a rural primary care physician for 22 years prior to joining the academic medical center. Martie Ross, JD, is a former practicing healthcare attorney and healthcare management consultant whose work over the last three decades has focused on rural healthcare delivery.

These executives are supported by two experienced project management specialists (one at 0.5 FTE and the other at 0.25 FTE), a full-time grants management specialist (who formerly served as a hospital chief financial officer), and a full-time executive assistant. The annual service fee is the sum of the actual costs to TUKHS' of employing these individuals (with the executives' compensation capped at Executive Level II).

TABLE 4.1 CCA Contract – Personnel Breakdown of UKHS Care Collaborative Association's Project Management line item in Table 4 (I/II) Each position is budgeted for 10 months in Year 1			
People/Category	Annual Salary	FTE/%	Total Year 1 Request
Executive - Robert Moser, MD	\$228,000	1.0	\$190,000.00
Executive - Martie Ross, JD	\$228,000	1.0	\$190,000.00
Project Management Specialist - Collin Cooper	\$213,000	.25	\$44,375.00
Project Management Specialist - Will Wilttrout	\$108,000	.5	\$45,000.00
Grants Management Specialist -Nora Loesche	\$193,440	1.0	\$161,200.00
Executive Assistant – to be filled	\$85,000	1.0	\$70,833.33
		Salary Total	\$701,408.33
Overhead (inclusive of benefits, taxes, and support services)	-	30	\$210,422.50
CCA comms and marketing expense	-	-	\$81,999.97
CCA legal and audit expense	-	-	\$75,000.00
Annual Service Fee			\$1,068,830.80

The State also will reimburse CCA for an executive's actual travel expenses associated with attending the annual CMS conference. The State has estimated this expense in the same manner it has calculated the expense for State employees. Also, the State will reimburse CCA for its executives' actual mileage in attending on-site meetings statewide, estimated at 6 trips per month at an average of 6 trips per month at an average of 280 miles per trip. These estimates are based on CCA executives' historic levels of travel for provider meetings, conference attendance, and

training sessions statewide.

TABLE 4.2 CCA Contract - Travel Expense				
<i>Breakdown of UKHS Care Collaborative Association's Project Management line item in Table 4 (II/II)</i>				
Purpose of Travel	Location	Item	Rate	Annual Cost
On-Site Meetings	State-wide	Mileage	0.725/mile x 20,000	\$14,500.00
CMS Conference	TBD	Travel Expenses	\$1,990 x 1 staff	\$1,990.00
Total				\$16,490.00

Budget Narrative – Initiative 1

Initiative 1 Programs	
1	Accountable Food is Medicine + Community Health Worker Deployment Program
2	Consumer-Facing Technologies Program
3	Behavioral Health Services Program
4	Integrated Care for Dual Eligible Beneficiaries Program
5	Mobile Cancer Screenings Program
6	Tribal Health Program

Table 5 summarizes the SF424A expenditure plan for Initiative 1: *Expand Primary and Secondary Prevention Programs* for each of the five performance years.

TABLE 5 Initiative 1 Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel	\$130,307.89	\$0	\$0	\$0	\$0
B. Fringe	\$56,735.75	\$0	\$0	\$0	\$0
C. Travel	\$21,220.00	\$0	\$0	\$0	\$0
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$12,572.90	\$0	\$0	\$0	\$0
F. Contract	\$25,041,725.46	\$24,739,386	\$23,916,386	\$17,729,182	\$26,054,720
H. Other	\$28,678.16	\$8,483,513	\$14,304,339	\$4,304,339	\$9,004,339
I. Direct Charges	\$25,291,240.16	\$33,222,899	\$38,220,725	\$22,033,521	\$35,059,059
J. Indirect Charges	\$50,523.77	\$868,688	\$868,688	\$868,688	\$868,688
Total	\$25,341,763.93	\$34,091,587	\$39,089,413	\$22,902,209 7	\$35,747,747

For the ten-month Budget Period 1 (December 31, 2025, to October 30, 2026), the State has budgeted the following amounts by category:

TABLE 5.A Year 1 Personnel – Salary and Wage					
Position	Annual Salary	Total FTE	Months	Total Year 1 Cost	Program
Bureau Director, Lainey Faulkner	\$104,247.63	0.15	5	\$6,515.48	1
Section Director, Vacant	\$73,800.00	0.2	4	\$4,920.00	1
CHW Unit Manager, Crystal Rodriguez	\$60,805.05	1	5	\$25,335.44	1
CHW Program Manager, Birgit Lemke	\$57,500.45	0.1	5	\$2,395.85	1
CHW Program Manager, Kelly Nightengale	\$57,533.00	0.9	5	\$21,574.88	1
CHW Specialist, Vacant	\$57,500.00	2	4	\$38,333.33	1
CHW Epidemiologist, Vacant	\$66,185.00	0.5	4	\$11,030.83	1
Ped Psych Program Manager	\$48,485.00	1	5	\$20,202.08	3D

The State also has budgeted for fringe benefits for the personnel identified above:

TABLE 5.B Year 1 Personnel – Fringe Benefits				
Fringe Benefit	Rate	Salary Requested	Total Year 1 Cost	Program
Fringe rate for Program 1 roles	20.94%	\$110,105.81	\$23,056.16	1
Insurance for Program 1 roles	\$14,817 x 1.80 FTE (accounts for months in role)		\$26,608.86	1
Fringe for Ped Psych Program Manager	35%	\$20,202.08	\$7,070.73	3D

The State has also budgeted for Travel, Supplies, and Indirect costs incurred by KDHE directly to support the personnel identified above and contracts identified below:

TABLE 5.C Year 1 Travel Expenses			
Purpose	Miles @ .725/mile	Total Year 1 Cost	Program
AFIM+CHW mileage	190 miles * 8 staff * 10 trips	\$11,020.00	1
AFIM+CHW vehicle rental, hotel, per diem	--	\$9,900.00	1
Ped Psych Program Manager Mileage and Tolls	--	\$300.00	3D

TABLE 5.E Year 1 Supplies				
Item	Rate	Units	Total Year 1 Cost	Program
AFIM+CHW standard supplies bundle	\$2,294.00	4.85	\$11,125.90	1
SAS	\$900.00	1	\$900.00	1
Mobile Tablet	\$360.00	1	\$360.00	1
Ped Psych Program Manager Supplies	\$187.00	1	\$187.00	3D

TABLE 5.G Year 1 Other Expenses		
Description	Total Year 1 Cost	Program
AFIM+CHW	\$51,625.65	1
Ped Psych	\$1,977.51	3D

TABLE 5.J Year 1 Indirect Charges		
Description	Total Year 1 Cost	Program
AFIM+CHW Indirect Costs	\$41,438.34	1
Pediatric Psych Indirect Costs	\$12,973.73	3D

The following tables detail by category the Year 1 expenses included in the contracts into which the State will enter to implement Initiative 1 (listed under “F. Contract” in Table 5). The number listed under “Program” corresponds to the number assigned in the chart above to each of the programs that comprise Initiative 1. The same convention is used for the other initiatives.

TABLE 5.F.1 Year 1 Personnel Expenses Included in Contracts					
Position	Annual Salary and Benefits	FTEs	Months	Total Year 1 Cost	Program
Remote Patient Monitoring RN	\$111,020.00	1	4	\$36,636.60	1
Behavioral Health Integration Director of Health Systems	\$130,921.25	0.45	4	\$19,638.00	3A
Telehealth Project Manager	\$120,559.00	1	3	\$30,140.00	3A
Nursing Facility Program Manager	\$90,405.00	1	4	\$29,833.65	3B
Behavioral Health Case Managers	\$58,427.00	25	6	\$730,338.00	3C
APRN .33 FTE per Hub x 6 Hubs	\$150,000.00	2	6	\$150,000.00	3C
SUD director	\$172,545.00	1	4	\$57,515.00	3E
Ambient RPM program manager	\$131,250.00	1	3	\$32,813.00	4

TABLE 5.F.2 Year 1 Travel Expenses Included in Contracts			
Purpose	Miles @ .725/mile	Total Year 1 Cost	Program
Telehealth Manager Travel	9,100	\$6,598.00	3A
Nursing Facility Program Manager Travel	13,793	\$10,000.00	3B
SUD Program Director Travel (mileage, per diem, airfare)	--	\$10,945.00	3E
RPM primary care engagement	8,410	\$6,097.00	4

TABLE 5.F.3 Year 1 Equipment Expenses Included in Contracts				
Description	Price per unit	No.	Total Year 1 Cost	Program
Care Coach technology	\$1,500,000.00	1	\$1,500,000.00	4

TABLE 5.F.4 Year 1 Supply Expenses Included in Contracts				
Description	Price Per Unit	Number	Total Year 1 Cost	Program
Remote patient monitoring units and glucose supplies	\$256.00	200	\$51,200.00	1
Remote monitoring clinic implementation kit	\$6,219	7	\$43,533.00	1
Telehealth Manager Supplies	\$1,776	1	\$1,776	3A
Nursing Facility Program Manager Supplies	\$2,984.00	1	\$2,984.00	3B
Computers, software, relays	\$10,656	6	\$63,936.00	3C
Data and cell phone communication	\$554.00	4	\$2,216.00	3C
Office supplies	\$165.00	4	\$660.00	3C
SUD Program Director Supplies	--	--	\$18,299.27	3E
AI remote ambient monitoring devices	\$629.50	50	\$31,475.00	4
Remote monitoring manager supplies	\$2,330.00	1	\$2,330.00	4

TABLE 5.F.5 Year 1 Third-Party Contracts (to include detailed breakdown with submitted contracts)		
Description	Total Year 1 Cost	Program
Implementing Healthcare Organizations	\$1,029,405.30	1
K-State CHW distribution and food infrastructure support	\$253,165.06	1
CHW Quick Start Training (UKCRI)	\$19,025.00	1
Sunflower Foundation: A-FIM evaluation and technical assistance	\$550,400.00	1
Computer-based cognitive behavioral health therapy	\$4,000,000.00	2A

LEAP! Alzheimer's Disease prevention	\$500,000.00	2B
Fitness and nutrition consumer-facing app	\$250,000.00	2D
Diabetes Prevention Program (DPP) video production	\$120,000.00	2E
Pharmacist DPP patient support services	\$7,395.84	2E
HealthTeamWorks: Integrated behavioral health technical assistance for clinics	\$420,000.00	3A
Integrated behavioral health clinic infrastructure payments	\$1,850,000.00	3A
Farley Health Policy Center: Integrated behavioral health learning collaboratives / e-learning module	\$150,000.00	3A
Telehealth Program Medical Director	\$3,000.00	3A
Myers and Stauffer	\$125,000.00	3B
Regional Nursing Facilities (x6)	\$1,200,000.00	3B
Behavioral health hub office leases	\$42,852.00	3C
University of Kansas Medical Center	\$717,364.95	3D
University of Kansas Center for Research Institute	\$113,103.00	3D
Kansas Chapter, Postpartum Support International	\$157,416.66	3D
Kansas Chapter, American Academy of Pediatrics	\$64,196.26	3D
SUD technology contracts	\$450,000.00	3E
SUD administrative contracts	\$48,400.00	3E
SUD provider contracts	\$160,418.00	3E
SUD marketing contracts	\$524,000.00	3E
PACE alternative care sites	\$500,000.00	4
PACE outreach/patient engagement	\$500,000.00	4
PACE center renovation	\$5,000,000.00	4
Mobile cancer screening program	\$144,000.00	5
Tribal Health	\$2,300,000.00	6

TABLE 5.F.6 Year 1 Other Expenses Included in Contracts

Description	Total Year 1 Cost	Program
Nursing Facility Program Manager / BH Specialist Training	\$5,829.00	3B
SUD other expenses	\$29,150.00	3E

TABLE 5.F.7 Year 1 Indirect Charges Included in Contracts

Description	Total Year 1 Cost	Program
K-State	\$25,316.51	1
Sunflower Foundation	\$55,040.00	1
UKCRI	\$1,902.50	1
CBCBT (10%)	\$400,000.00	2A
Sunflower Foundation	\$243,963.80	3A
KDADS, BH in NF (15.6%)	\$7,588.88	3B
University of Kansas Medical Center (10%)	\$71,736.49	3D
University of Kansas Center for Research Institute (10%)	\$11,310.30	3D
Kansas Chapter, Postpartum	\$15,741.67	3D

Support International (10%)		
Kansas Chapter, American Academy of Pediatrics (10%)	\$6,419.63	3D
BHNC/SUD Network	\$129,272.73	3E

The following discussion details Initiative 1 expenditures by individual program.

Program 1: Accountable Food Is Medicine and Community Health Worker (CHW) Deployment Program (A-FIM). In Year 1, the KDHE will work with rural clinics and K-State University Research and Extension to recruit, train, and place at least 10 CHWs throughout Kansas. 1-2 CHWs will be placed in up to 7 rural implementing healthcare organizations, which will be selected via RFA and sub-awarded by KDHE based on a variety of factors, including readiness as determined by KDHE's Clinic Readiness Assessment. Clinics will determine their own budgets in collaboration with KDHE, budgeting for personnel, supplies, infrastructure, and more. An additional 5 CHWs will be hired by K-State Research and Extension and placed in rural extension offices throughout the state. K-State University Research and Extension plans to hire more CHWs as well as registered dietitians and ag/food system specialists to support A-FIM in future years.

The State will ensure all hired CHWs will be trained and certified through a state-approved CHW Core Competency Course taught by approved education providers. Additionally, KDHE will ensure all CHWs receive specialized A-FIM training and ongoing technical assistance to ensure successful integration and sustainability. In Year 1, KDHE will contract with the University of Kansas Center for Research Inc for Department of Family Medicine & Community Health via task order to provide Quick Start CHW training; in Year 2, KDHE anticipates competitively selecting additional entities to provide CHW training.

KDHE's Bureau Director for Health Promotion will devote 0.15 FTE to overseeing this work. KDHE staff, including 2 CHW Specialists and an Epidemiologist, will support participating clinics and community-based organizations in planning and implementation of program objectives. KDHE will work with K-State Research and Extension to plan the establishment of A-FIM hubs to support operations regionally and within local communities. Additionally, K-State Research and Extension will work with KDHE and other relevant partners to support client education, training, and promotion of the A-FIM program.

KDHE will contract with CCA to provide remote patient monitoring services for some patients/clients participating in the A-FIM program. The budget includes staff time for a remote monitoring nurse and the purchase of remote patient monitoring units and glucose supplies.

Finally, KDHE will sub-award Sunflower Foundation to provide AFIM technical assistance and coordinate the evaluation of this project. Sunflower's resources will allow KDHE to work with partners to effectively evaluate A-FIM implementation and progress. Sunflower will contract with KUMC to engage Dr. Kristina Bridge for statewide evaluation and technical assistance, contract with UKCRI to engage Dr. Susan Harvey for process evaluation, and contract with University of Oklahoma Health Campus to engage Dr. Mariana Wetherhill, one of the nation's leading FIM experts at the University of Oklahoma, to bring best practices to the Kansas program.

Program 2: Consumer-Facing Technology Program. This program includes the acquisition and deployment in rural communities of five technology-based primary and secondary prevention and wellness interventions. KDHE will sub-award CCA to execute these programs and/or pass through funds.

A. Computerized Cognitive Behavioral Health Therapy. CCA will sub-award the University of Kansas Medical Center Department of Psychiatry to deploy Thrive, a computerized cognitive behavioral health therapy (CCBHT) program, in all rural Kansas counties. Payments under the contract will total \$4M in Year 1, including personnel, technology licensing and platform deployment, outreach, and evaluation.

The State selected Thrive over other computerized cognitive behavioral health therapy solutions because it is uniquely positioned for implementation in rural communities. First, Thrive has demonstrated the largest and broadest clinical effects across randomized controlled trials, producing approximately 50% greater improvement in depressive symptoms (PHQ-9) compared to the other leading digital intervention (Schure, 2019; Stuart, 2022; Rejoyn Clinician Brief Summary).¹

Second, Thrive is the only solution that has undergone a formal rural adaptation and has been shown to be effective in a large randomized controlled trial conducted specifically among rural

¹ Schure MB, Lindow JC, Greist JH, Nakonezny PA, Bailey SJ, Bryan WL, Byerly MJ. Use of a Fully Automated Internet-Based Cognitive Behavior Therapy Intervention in a Community Population of Adults With Depression Symptoms: Randomized Controlled Trial. *J Med Internet Res*. 2019;21(11):e14754. Epub May 18, 2019. doi: 10.2196/14754; PMID: PMC6887812; Stuart R, Fischer H, Leitzke AS, Becker D, Saheba N, Coleman KJ. The Effectiveness of Internet-Based Cognitive Behavioral Therapy for the Treatment of Depression in a Large Real-World Primary Care Practice: A Randomized Trial. *Perm J*. 2022 Sep 14;26(3):53-60. doi: 10.7812/TPP/21.183. Epub 2022 Aug 1. PMID: 35939620; PMID: PMC9683746. The referenced Rejoyn Clinician Brief Summary is available at <https://www.rejoynhcp.com/Clinician-Brief-Summary.pdf>

residents (Schure, 2019). Thrive includes regular symptom monitoring and automated alerts for safety concerns or lack of clinical improvement, which is critical in low-resource rural settings. Third, the owners of Thrive have extensive experience partnering with academic teams and large clinical delivery systems, including long-standing collaborations with academic digital mental health programs and health systems. This experience is essential to implementation at scale, integration with clinical workflows, and rigorous evaluation—key requirements of this initiative.

Fourth, Thrive’s built-in utilization, symptom trajectory, and safety monitoring capabilities directly support the initiative’s core focus on evaluation, data, and continuous improvement. These features enable real-time assessment of engagement, clinical response, and safety signals at both the individual and population levels, supporting continuous quality improvement, performance monitoring, and data-driven refinement of implementation strategies.

B. LEAP! CCA will sub-award The University of Kansas Alzheimer’s Disease Research Center to expand access to LEAP! (Lifestyle Empowerment for Alzheimer’s Prevention) in rural Kansas. LEAP! is an evidence-based, virtually- and digitally-delivered brain health intervention program focused on modifiable risk factors for dementia. The platform provides education on physical activity, nutrition, cognitive stimulation, social engagement, stress management, and sleep health through an accessible online interface.

The Alzheimer’s Disease Research Center has offered the LEAP! program for more than a decade, reaching over 10,000 participants across 46 Kansas counties and 23 states, with strong engagement and satisfaction following its transition to a digital delivery model in 2020. Compared with large, resource-intensive trials such as FINGER and POINTER, LEAP! is designed to be more efficient, lower-cost, and less provider-dependent, prioritizing scalability and real-world implementation in community settings.

Despite this demonstrated success, LEAP!’s current reach within rural Kansas remains limited, with participation to date concentrated largely in the Kansas City metropolitan area and an estimated 300 participants identified from rural ZIP codes in the past 6 years. This gap reflects constraints in technology infrastructure, rural-specific outreach, and referral pathways rather than program efficacy or demand. Existing funding supports program delivery but does not cover the development of new capabilities required for rural transformation. Year 1 funding of \$500,000 will support salary and benefits for the dedicated project team (\$101,624) technology and content development expenses (\$120,000), and targeted outreach activities to engage rural providers and promote LEAP! in rural communities, and ongoing education and support for rural providers.

C. My ALZ Journey. CCA will contract with the Alzheimer's Association to access this application for rural Kansans recently diagnosed with Alzheimer's Disease and their care partners. It provides early-stage disease education, community resources, care consultations, and planning tools. This project will begin in Year 2, with budget funding technology licensing fees, program implementation, and provider engagement.

D. Fitness/Nutrition/Lifestyle Applications. The State received proposals from multiple vendors for deployment of such applications. To ensure Kansas invests RHT Program funds efficiently, CCA will issue a Request for Proposals in for deployment of fitness/nutrition/lifestyle applications in rural Kansas communities. The State has budgeted \$250,000 for this purpose.

E. Asynchronous Diabetes Prevention Program. CCA will develop and implement an A-DPP to which rural providers and CHWs working in rural communities can refer qualifying patients. The Year 1 budget includes \$120,000 for development and editing of 12 patient education video modules consistent with the Centers for Disease Control's DPP requirements. The budget also includes \$7,396 to reimburse contracted rural pharmacists who provide care management visits for DPP participants, as such services are not otherwise reimbursable by insurance. The payment per visit will be \$51.36, equivalent to Medicare reimbursement for CPT 99212, assuming 144 patient visits in Year 1. CCA will provide additional services for DPP participants, including virtual visits and remote patient monitoring, but these services will be funded through other sources.

Program 3: Behavioral Health Services Program. Projects A, B, C, and E here will be operated by KDADS through its subaward with KDHE. Project D will be executed directly by KDHE.

A. Behavioral Health Integration in Primary Care. KDADS will sub-award the Sunflower Foundation to recruit 74 rural primary care practices to participate in on-site training to evaluate implementation of integrated behavioral health services in their clinics. For more than a decade, the Sunflower Foundation has been actively engaged with providers in supporting behavioral health integration in primary care in Kansas, most recently partnering with the Farley Center at the University of Colorado and HealthTeamWorks to provide an Integrated Care Learning Network (ICLN), an on-ramp for training and technical assistance for Kansas clinicians and administrators working to provide integrated, whole-person care to their communities. Both organizations have deep experience developing the ICLN framework and supporting practice implementation, and are well-positioned to expand the program using RHTP funding.

Sunflower will allocate 45% of their Director of Health Systems, budgeted at \$19,638 to operate the project for 4 months in Year 1. Sunflower will contract with Farley Health Policy Center and HealthTeamWorks to expand the state's existing ICLN to 74 clinic sites. Farley Health Policy

Center will continue to serve as the ICLN lead advisor, and HealthTeamWorks will continue serving as the network's lead technical assistance provider. Projected contract expenses total \$570,000. Each clinic that formally commits to program participation through execution of a Memorandum of Understanding will receive a one-time \$25,000 infrastructure payment upon meeting specified milestones including, but not limited to, dedicated clinical and administrative staff time, formal evaluation of clinic operations, clinical and administrative staff attendance at training sessions and other interactions with training team, documented workflow modifications, policy and procedure development and implementation, development and distribution of patient education materials, technology upgrades, and data collection and reporting. The State has budgeted \$1.85M for these infrastructure payments.

Anticipating at least some of the clinics participating in this process will elect to make these services available through a partnership with a virtual provider (as opposed to hiring or re-assigning existing staff for this role), KDADS will sub-award CCA to expand its existing virtual program by adding a telehealth project manager with associated travel and supply expenses, and to sub-contract Medical Director expertise to provide on-site boot camp training with primary care providers.

B. Behavioral Health Services in Nursing Facilities. The Kansas Department for Aging and Disability Services will execute the development, implementation, and evaluation of an embedded behavioral health staffing model for rural nursing facilities. The Kansas Department for Aging and Disability Services (KDADS) will hire an RHTP Nursing Facility Program Manager (with annual salary and benefits of \$90,405 for 4 months in Year 1) to facilitate this program. The State has budgeted \$10,000 for travel expenses for this individual and \$2,984 for supplies. These supplies include the same items at the same costs as those listed in Table 2.5 for KDHE staff. Six rural NFs, selected via competitive application process, will be sub-awarded by KDADS up to \$200,000 per year, paid as \$175 per eligible resident per Medicaid day to provide non-reimbursable services to manage complex behavior. KDADS will contract Myers and Stauffer for \$125,000 in Year 1 to conduct a nationwide analysis of existing similar Medicaid add-ons and develop a detailed plan to implement an add on in Kansas. The results of this project will inform the State's decision whether to pursue a Medicaid nursing facility behavioral health add-on payment.

C. Emergency Department Behavioral Health Intervention Hubs. KDADS will sub-award Healthsource Integrated Solutions, a subsidiary of the Association of Community Mental Health Centers of Kansas (ACMHCK, a group of 26 community mental health centers with a combined staff of over 4,500 providing mental health services across every county of the State) to establish behavioral health intervention hubs at six regional emergency departments. The Year 1 budget was estimated based on a hub staffing model with 4.2 FTE case managers (with salary and benefits of

\$58,427 each annually, assuming these positions will be in place for six months in Year 1) and the six hubs will share two psychiatric APRNs (with salary and benefits of \$150,000 each annually, assuming these positions will be in place for six months in Year 1). In addition to personnel, the contract will include funds for the lease of space at the hospitals and the standard technology and supply bundle for each FTE.

D. Pediatric Psychiatric Access KDHE has budgeted \$1.2 million in Year 1 to operate the Pediatric Psychiatric Access program. KDHE has budgeted \$27,272.81 to cover salary and fringe for a Program Manager, in-place for 5 months of Year 1. KDHE has also budgeted \$300.00 to cover ~400 miles of in-state travel to attend project meetings, training events, and outreach activities, as well as \$187.00 for Year 1 supplies.

KDHE will execute contracts/task orders with existing partners at the University of Kansas Medical Center (UKMCRI) and the University of Kansas Center for Research Institute (UKCRI). UKMCRI will be contracted to establish pediatric and perinatal mental health teams (PMHT) that provide training and technical assistance to PCPs to diagnose, treat, and refer children, adolescents, and perinatal individuals with behavioral health conditions. UKCRI will be contracted to conduct a rural provider needs assessment, provide rural-ready educational and clinical practice materials, and support in-person workforce development planning.

KDHE will also sub-award the Kansas Chapter of Post-Partum Support International (PSI) and the Kansas Chapter of the American Academy of Pediatrics (KAAP). PSI will be sub-awarded to facilitate quality improvement activities and training related to perinatal behavioral health screening. KAAP will be sub-awarded to conduct practitioner outreach, recruitment, training, and sustainability activities.

E. Statewide SUD Referral and Stabilization Network. The State has budgeted \$1.4 million in Year 1 for KDADS to contract with the Behavioral Health Network of Kansas to operate the statewide network. The contract will include \$57,515.00 for salary and benefits for a program director staffed for 4 months in Year 1. The contract also will include funds for technology, administrative, provider, and marketing contracts to operate the network successfully.

Program 4: Integrated Care for Dual Eligible Beneficiaries Program. The State has budgeted \$7.5M in Year 1 for KDADS to sub-award two PACE providers currently operating in Kansas to significantly expand the availability of PACE programs for qualifying dual eligible beneficiaries in rural Kansas. These PACE providers also will engage in concentrated patient enrollment efforts in rural areas.

KDADS will sub-award CCA to expand its existing remote patient monitoring program to provide AI ambient monitoring for dual eligible beneficiaries for whom PACE is not an option, leveraging shared remote monitoring staff and offset by billable services after Year 1. The Year 1 budget includes ~\$30,000 for the purchase of 50 monitoring devices and \$2,330 for related supplies (based on vendor-supplied pricing), mileage for 8,410 miles to travel to primary care sites and patient homes, and salary and benefits for a Nurse Program Manager at \$131,250 annually, assuming this position will be in place for three months in Year 1.

Program 5: Mobile Cancer Screening Program. The program extends the use of the existing University of Kansas Cancer Center's mobile medical unit to 10 rural communities annually with limited or no access to dermatologists or urologists for routine cancer screenings. The average annual expense to provide non-reimbursable screenings in a rural community is \$14,400, which is based on typical salary and benefits for the screening team (\$10,162), average travel expenses for the screening team (\$3,588), and the daily rate for the mobile medical unit operations (\$650) for a total Year 1 budget of \$144,000. KDHE will sub-award CCA to contract KUMC for this project.

Program 6: Tribal Health Program. The State has budgeted \$2.3M for a grant program to be administered by KDHE with the funds to be used by the federally recognized tribes in Kansas to develop and implement primary and secondary prevention programs associated with their rural clinics and communities. Each sovereign tribe will develop its own plan for use of these funds, to support local initiatives identified that include primary and secondary prevention programs, with the goal to improve the overall health of their communities. Proposed programs and activities will be presented to the State for approval prior to implementation to ensure compliance with RHT Program requirements. KDHE has budgeted to sub-award an equal amount of funds to 3 entities covering Kansas' 4 sovereign Tribes: The Prairie Band Potawatomi Nation, the Kickapoo Tribe in Kansas, and White Cloud Health Center (which covers the Sac and Fox Nation of Missouri in Kansas and Nebraska and the Iowa Tribe of Kansas and Nebraska).

Budget Narrative – Initiative 2

Initiative 2 Programs	
1	Regional Partnerships Grant Program
2	REH Conversion/Transformative Capital Investment Grant Program
3	Revenue Improvement Program
4	Anchor Hospital Advancement Program
5	Mobile Integrated Health Pilot Program
6	Rural Primary Care – Public Health Integration Program

Table 7 summarizes the SF424A expenditure plan for *Initiative 2: Secure Local Access To Primary Care* for each of the five performance years.

TABLE 7 Initiative 2 Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel/Fringe	\$15,938.77	\$0	\$0	\$0	\$0
C. Travel	\$0	\$0	\$0	\$0	\$0
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$0	\$0	\$0	\$0	\$0
F. Contract	\$97,247,153.69	\$18,150,531	\$15,698,109	\$14,961,243	\$15,726,051
H. Other	\$0	\$47,126,200	\$44,126,200	\$47,000,000	\$32,000,000
I. Direct Charges	\$97,263,092.46	\$65,276,731	\$59,824,309	\$61,961,243	\$47,726,051
J. Indirect Charges	\$0	\$0	\$0	\$0	\$0
Total	\$97,263,092.46	\$65,276,731	\$59,824,309	\$61,961,243	\$47,726,051

For the ten-month Budget Period 1 (December 31, 2025, to October 30, 2026), the State has budgeted the following amounts by category:

TABLE 7.A Year 1 Personnel – Salary and Wage					
Salary and Wages	Annual Salary	Total FTE	Months	Total Year 1 Cost	Program
Director, Bureau of Public Health: Alyson Dalrymple	\$112,737.00	0.2	4	\$7,515.80	6
Director, Local Public Health Program: Cristi Cain	\$126,344.55	0.2	4	\$8,422.97	6

The following tables detail by category the Year 1 expenses included in the contracts into which the State will enter to implement Initiative 2 (listed under “E. Contract” in Table 7).

TABLE 7.F.1 Year 1 Personnel Expenses Included in Contracts					
Position	Annual Salary and Benefits	FTEs	Months	Total Year 1 Cost	Program
Credentialing Specialist	\$125,000.00	1	4	\$41,666.67	3A
Credentialing Program Manager	N/A (billed hourly)	20 hrs. per month	4	\$10,800.00	3A
Mobile Integrated Health Program Director	\$137,000.00	1	3	\$34,375.00	5
Primary Care/Public Health Integration Team	N/A	N/A	N/A	\$117,597.00	6

Table 7.F.2 Year 1 Travel Expenses Included in Contracts			
Purpose	Rate	Total Year 1 Cost	Program
Centralized Credentialing Org: Site visit travel	\$750/site	\$7,500.00	3B
Centralized Credentialing Org: Conference attendance	N/A	\$1,800.00	3B
Centralized Credentialing Org: Mileage reimbursement	.725/mile	\$840.00	3B
Primary Care Public Health Integration - site visits	.725/mile	\$8,000.00	6

Table 7.F.4 Year 1 Supply Expenses Included in Contracts				
Description	Cost	No.	Total Year 1 Cost	Program
Centralized Credentialing Organization: Laptop and mobile device	\$2,330.00	1	\$2,330.00	3B
Centralized Credentialing Organization: Office supplies	\$165.00	1	\$165.00	3B
Centralized Credentialing Organization: Printing/marketing materials	\$1,200.00	1	\$1,200.00	3B
Centralized Credentialing Organization: Software	\$710.00	1	\$710.00	3B
Primary Care Public Health Integration – supplies	\$5,270.00	1	\$5,270.00	6

Table 7.F.5 Year 1 Third-Party Contracts (to include detailed breakdown with submitted contracts)		
Description	Total Year 1 Cost	Program
Regional Partnership Grant Program	\$49,969,410.72	1
REH Conversion/Transformational Capital Investment Grant Program	\$31,279,891.30	2
Statewide Commercial Rate Analysis	\$1,500,000.00	3A
Revenue Cycle Technical Support Organization Contracted Staff	\$1,800,000.00	3B
Rural CFO Training and Technical Assistance	\$150,000.00	3B
Payer Dashboard development and distribution	\$221,400.00	3B
Credentials Verification Organization	\$1,339,998.00	3B
Direct-to-Employer Consulting Services	\$114,000.00	3C
Anchor Hospital on-demand subject matter experts	\$414,000.00	4
Anchor Hospital artificial intelligence (AI)/EHR optimization	\$9,800,000.00	4
Primary Care/Public Health Integration Operating Hubs	\$150,000.00	6
Primary Care/Public Health Integration Data Support	\$130,000.00	6
Primary Care/Public Health Integration Consulting Support	\$20,000.00	6

TABLE 7.F.6 Year 1 Other Expenses Included in Contracts		
Description	Total Year 1 Cost	Program
Centralized Credentialing Organization Credentialing curriculum	\$1,200	3B
Certified Medical Assistant Educational Award (5-year rural practice commitment)	\$125,000	5

The following discussion details Initiative 2 expenditures by individual program.

Program 1: Regional Partnership Grant Program (RPGP). KDHE will sub-award competitive grant applicants pursuing collaborative arrangements with one or more Eligible Providers (defined as

Kansas hospitals, physician practices (including solo practitioners), rural health clinics, federally qualified health centers, local health departments, Native American Sovereign Tribe healthcare facilities, certified community behavioral health clinics, licensed substance use disorder facilities, and licensed long-term care facilities located in rural areas) for the purpose of transforming rural healthcare delivery within a region in a sustainable manner. The State is awarding grants totaling the following amounts each year:

Table 8 RPGP Funds Available by Year <i>Aligns with Table 7.E.5</i>	
Year	Amount
Year 1	\$50.0M
Year 2	TBD
Year 3	TBD
Year 4	TBD
Year 5	TBD

The draft RPGP Request for Applications and application form detailing grant requirements, evaluative criteria for award selection, and grantee reporting requirements were developed with input from stakeholders. KDHE will administer RPGP (including solicitation of applications, evaluation of applications, funds administration, performance monitoring, and evaluation), the costs of which are included in KDHE's administrative budget detailed above.

Program 2: REH Conversion/Transformative Capital Investment Grant Program (REH-CAP Program). KDHE will sub-award (1) facilities that have converted or are in the process of converting to Rural Emergency Hospital (REH) status to fund necessary facility renovations and (2) other Eligible Providers seeking to make capital investments to transform the manner in which they serve their communities. The State is awarding grants totaling the following amounts each year:

Table 9 REH-CAP Funds Available by Year <i>Aligns with Table 7.E.5</i>	
Year	Amount
Year 1	\$31.3M
Year 2	TBD
Year 3	TBD
Year 4	TBD
Year 5	TBD

The draft REH-CAP Program Request for Applications and application form detailing grant requirements, evaluative criteria for award selection, and grantee reporting requirements were developed with input from stakeholders. KDHE will administer the REH-CAP Program (including solicitation of applications, evaluation of applications, funds administration, performance

monitoring, and evaluation), the costs of which are included in KDHE's administrative budget detailed above.

Program 3: Revenue Improvement Program. There are four projects under this program, each of which will be executed via sub-award with CCA.

A. Commercial Rate Analysis. CCA will contract with PYA, PC, a national healthcare consultancy, to perform a statewide rural provider commercial rate analysis. PYA recently performed similar analyses for rural providers in other states, including North Dakota and Wisconsin, having developed a reliable methodology and efficient processes to produce usable work product in a timely manner. The State has budgeted \$1.5 million in Year 1 for the commercial rate analysis based on the proposal received from PYA to perform this work.

B. Revenue Cycle Technical Support Organization. The State has budgeted for CCA to enter into a subrecipient agreement with Healthworks KHA Foundation, Inc. (Healthworks) to deliver revenue cycle technical support for rural providers. Healthworks will conduct an RFP to identify and contract with a qualified entity for \$1.8 million in Year 1 to provide a team of experts in medical coding, billing, reimbursement, managed care contracting, and regulatory compliance to directly support rural providers with revenue cycle operations and performance improvement. This team also will pursue payment recovery on denied and unpaid claims on providers' behalf. Healthworks also will conduct an RFP to identify and contract with a qualified entity for \$221,400 in Year 1 to develop and distribute a payer dashboard to assist rural providers track performance against state and national benchmarks to support performance improvement. Finally, Healthworks will conduct an RFP to identify and contract with a qualified entity in Year 1 for \$150,000 to develop and present rural CFO training, including both in-person, virtual, and on-demand sessions, as well as development and distribution of performance dashboards and other reference materials.

B. Centralized Credentialing. The State has budgeted for CCA to enter into a subrecipient agreement with Healthworks to serve as the project sponsor for the centralized credentialing program, including regular interactions with providers and payers to address questions and concerns and to facilitate process improvements. This budget includes a credentialing curriculum and a full-time credentialing specialist with the standard technology and supply bundle and travel reimbursement at a cost of \$68,212 in Year 1. Healthworks will conduct an RFP to identify and contract with a qualified entity to provide payer credentialing services for rural providers.

C. Direct-to-Employer Consulting Services. The State has budgeted \$114,000 for CCA to competitively secure consulting services to develop and execute on a plan to educate rural providers and employers regarding direct-to-employer contracts.

Program 4: Anchor Hospital Advancement Program. KDHE will sub-award CCA to contract with the Anchor Hospital Advancement Program (AHAP), a joint venture formed by Kansas’ urban not-for-profit health systems to provide specified operational support services for Kansas’ rural anchor hospitals (i.e., ten larger rural facilities providing specialist services on a regional basis) to strengthen the state’s overall rural health delivery system. The Year 1 budget for these operational support services includes the following:

TABLE 10 AHAP Support Services <i>Corresponds with the Anchor Hospital line items included in Tables 7.F.1 and 7.F.5</i>	
Operational Support Service	Annual Budget
On-Demand Subject Matter Expert Resources	\$414,000
Artificial Intelligence and EHR Implementation and Optimization	\$9.8M

CCA will distribute funds to AHAP as it secures contractual commitments from anchor hospitals to utilize specified services and to report relevant data regarding the use of those services.

Program 5: Mobile Integrated Health Pilot Program. The State will sub-award CCA to train, place, and supervise certified medical assistants (CMAs) in five rural communities to perform home visits for Medicaid beneficiaries in those communities. In Year 1, the budget will include funds to train five individuals in a CMA program at a Kansas technical college, including \$25,000 per student in educational award (subject to a 5-year rural practice commitment). The Year 1 budget also includes \$34,375.00 for CCA to hire a Program Manager at an annual salary of \$137,500.00, in place for 3 months in Year 1.

The sub-award will fund salaries and benefits for the five CMAs who will be making Medicaid home visits in the selected rural communities, as there is no reimbursement currently available for these services. The annual salary for the CMAs will be based on an hourly rate of \$32.45, which is higher than the average rate of CMA compensation in Kansas as an incentive to work providing services in patients’ homes. The State Medicaid program will evaluate whether to establish reimbursement for these services based on pilot results. If such reimbursement becomes available during the term of the RHT Plan, the funding for the CMA salaries will be reduced accordingly.

Program 6: Pathways to Integration - Rural Primary Care/Public Health Integration Program. KDHE will sub-award the University of Kansas Medical Center Department of Population Health to support Pathways to Integration (P2I), a multi-year initiative to improve

access to primary care and preventive services in rural Kansas by strengthening integration between local health departments and primary care providers. The project will be implemented across six rural regions in three phases: (1) assessment and planning, (2) implementation and right-sizing, and (3) maintenance and sustainability.

During the assessment phase, regional partners will identify shared population-health priorities, map existing services and capabilities, assess readiness for collaboration, and develop formal partnership agreements. Implementation activities will focus on co-locating services where appropriate, strengthening referral and care coordination practices, and developing or enhancing data-sharing infrastructure to support shared decision-making, continuous quality improvement, and accountability for outcomes.

P2I will provide centralized backbone support, technical assistance, and evaluation to guide partners and ensure progress. Allocated KDHE and KUMC project staff and consultants will support partner engagement, curate adaptable tools and resources, and facilitate peer learning across regions. For Year 1, salary and benefits for KDHE project staff will total \$15,938.77, assuming these positions will be in place for six months in Year 1. An additional \$300,000 in Year 1 will include \$130,000 for KUMC to contract with KHA for the creation of a data sharing infrastructure; \$20,000 to fund KUMC to contract with consulting support as needed, and \$150,000 for KDHE to directly sub-award its first regional partner. KUMC's budget also personnel at \$117,597 and supplies/travel at \$13,270 in Year 1.

Budget Narrative – Initiative 3

Initiative 3 Programs	
1	Physician Pipeline Program
2	Education and Training Program
3	Recruitment and Retention Program
4	Career Exploration Program

The following table summarizes the SF424A expenditure plan for Initiative 3: *Build a Sustainable Rural Workforce* for each of the five performance years.

TABLE 11 Initiative 3 Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel/ Fringe	\$0	\$0	\$0	\$0	\$0
C. Travel	\$0	\$0	\$0	\$0	\$0
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$0	\$0	\$0	\$0	\$0
F. Contract	\$12,184,654.71	\$12,785,133	\$13,626,868	\$13,636,868	\$15,626,868
H. Other	\$0	\$7,857,400	\$7,857,400	\$7,857,400	\$7,857,400
I. Direct Charges	\$12,184,654.71	\$20,642,533	\$21,484,268	\$21,494,068	\$23,484,268
J. Indirect Charges	\$0	\$0	\$0	\$0	\$0
Total	\$12,184,654.71	\$20,642,533	\$21,484,268	\$21,494,068	\$23,484,268

The following tables detail by category the Year 1 expenses included in the contracts into which the State will enter to implement Initiative 3 (listed under “E. Contract” in Table 11).

TABLE 11.F.1 Year 1 Personnel Expenses Included in Contracts					
Position	Annual Salary and Benefits	FTE	Months	Total Year 1 Cost	Program
Kansas Bridging Program Manager	\$81,000.00	1	6	\$40,500.00	1B
Health Science Educational Award Program Manager	\$120,000.00	1	6	\$60,000.00	2A
Dental Educational Award Executive Director	\$180,000.00	0.1	5	\$7,500.00	2C
Dental Educational Award Comms Director	\$72,000.00	0.175	5	\$5,250.00	2C
Dental Educational Award Asst. Exec Director	\$103,800.00	0.1	5	\$4,325.00	2C
Dental Educational Award Financial Manager	\$61,000.00	0.125	5	\$3,177.08	2C
Dental Educational Award Membership Personnel	\$50,000	0.15	5	\$3,125.00	2C
RN Nurse Trainer	\$118,750.00	1	4	\$39,583.33	2D
Mobile Simulation Manager	\$93,750.00	1	6	\$46,875.00	2D

TABLE 11.F.2 Year 1 Travel Expenses Included in Contracts			
Purpose	Miles @ .725/mile	Total Year 1 Cost	Program
Dental Educational Award Travel	--	\$2,500.00	2C
Mobile Simulation Vehicle	32,828	\$23,800.30	2D

Table 11.F.3 Year 1 Equipment Expenses Included in Contracts				
Description	Price Per Unit	No.	Total Year 1 Cost	Program
Modified Recreational Vehicle	\$750,000.00	1	\$750,000.00	2D
High-Fidelity Manikin	\$117,615.00	1	\$117,615.00	2D
Trauma Limbs	\$40,000.00	1	\$40,000.00	2D

Table 11.F.4 Supply Expenses Included in Contracts				
Description	Price Per Unit	Number	Total Year 1 Cost	Program
KHA Program Managers Supplies	\$2,984.00	3	\$8,952.00	1-4
KBP Tracking Software	\$4,045.00	1	\$4,045.00	1B
Dental Educational Award Brochures	\$1,000.00	1	\$1,000.00	2C
Virtual Headsets	\$799.00	8	\$6,392.00	2D
Software and 15 Training Modules	\$140,060.00	1	\$140,060.00	2D
Medical Training Supplies	\$90,000.00	1	\$90,000.00	2D

Table 11.F.5 Year 1 Third-Party Contracts (to include detailed breakdown with submitted contracts)		
Description	Total Year 1 Cost	Program
Rural Residency Programs – KUMC and anchor hospitals	\$2,138,590.00	1A
Health Professions Training Grants	\$3,000,000.00	2B
Clinical Instructor and Preceptor Payments	\$500,000.00	3C
Nurse Residency Curriculum	\$215,000.00	3B
Behavioral Health Apprenticeships	\$63,600.00	3D
HOSA AHEC Direct Costs	\$122,727.00	4A
K-12 Career Technical Education	\$216,000.00	4B
Regional Career Expos	\$40,000.00	4D
Healthworks Project Management	\$285,447.00	1C, 3B, 3C, 4B, 4D

TABLE 11.F.6 Year 1 Other Expenses Included Under Contract		
Description	Total Year 1 Cost	Program
Kansas Bridging Program Expansion Direct Costs	\$545,955.00	1B
Medical Student Rotation Housing Stipends	\$250,000.00	1C
Health Professions Educational Award (subject to 5-year rural practice commitment)	\$2,600,000.00	2A
Dental Educational Award (subject to 5-year rural practice commitment)	\$650,000.00	2C

TABLE 11.F.6 Year 1 Indirect Expenses Included Under Contract		
Description	Total Year 1 Cost	Program
Kansas Bridging Program Expansion Indirect Costs	\$59,455.00	1B
BH Apprenticeships Indirect	\$1,860.00	3D
HOSA Indirect Costs	\$12,273.00	4A

The State has budgeted for CCA to contract with Healthworks to provide project management services for several workforce initiatives at a cost of \$285,447 in Year 1. The State selected Healthworks based on its extensive experience with Kansas healthcare workforce-related initiatives and its strong reputation as a trusted partner statewide.

The following discussion details Initiative 3 expenditures by individual program.

Program 1: Physician Pipeline Program. There are three projects under this program, each executed via KDHE's subrecipient agreement with CCA.

A. Rural Residency Programs. The State has budgeted in Year 1 for CCA to contract with AHAP (\$2,236,590.00) to plan for the establishment of rural residency pathways in four high-need specialties: OB/GYN, Pediatrics, Family Medicine, and Psychiatry. Two additional high-need specialties will be added in future years: General Surgery and Orthopedics. AHAP will establish an Office of Rural Residency Programs to coordinate with the University of Kansas School of Medicine – Kansas City, the University of Kansas School of Medicine – Wichita, and the University of Kansas School of Medicine – Salina to evaluate existing rural residency planning initiatives, to prioritize needs, and to allocate funds in a manner that maximizes statewide opportunities.

B. Expansion of the Kansas Bridging Program. CCA will sub-award the University of Kansas Medical Center Institute for Community Engagement to expand the existing Kansas Bridging Program to enhance and expand incentives for physicians completing their residencies to practice in Kansas. This includes \$550,000.00 to fund tracking software and existing slots and new slots in high-need specialties. The Year 1 budget also includes \$40,500 for a full-time program manager, assuming this position will be in place for six months in Year 1 and \$59,455.00 in indirect costs.

C. Medical Student Rural Rotation Housing Assistance. CCA will make \$250,000 in payments to hospitals for actual expenses incurred in securing housing and making minor renovations to ensure it meets specific requirements for medical students performing rotations in their communities. None of the funds will be used to provide a resource of value directly to an individual that would cause the 5-year rural practice commitment to apply.

Program 2: Education and Training Program. This program includes four projects.

A. Health Professions Service Educational Award Program. KDHE will sub-award the Kansas State Board of Regents to establish and administer Undergraduate/Technical College educational award in the amount of \$5,000 per year for 220 students and Post-Graduate Health Professions

Educational Award of \$15,000 year for 100 students. Recipients will be subject to a five-year rural practice commitment. Program administration is budgeted at 1.0 FTE with annual salary and benefits of \$120,000, assuming this position will be in place for six months in Year 1. The Board presently administers a similar educational award program for nursing students.

B. Health Professions Training Grants. KDHE will sub-award KBOR \$3M in Year 1 to administer a statewide competitive grant program to expand existing allied health training programs. KBOR is in the process of developing the request for application and application form for these grants.

C. Rural Dentistry Educational Award. KDHE will sub-award the Kansas Dental Association (KDA) to administer the rural dentistry educational award program. A total of \$650,000 in educational awards will be available in Year 1, with 10 awards of \$50,000 each for dental students and 15 awards of \$10,000 each for hygiene students. Recipients will be subject to a five-year rural practice commitment. KDHE will pay KDA \$26,877.08 in Year 1 for 0.65 FTE of staff for 5 months and travel/supplies for associated staff.

D. Mobile Simulation Lab. KDHE will contract with CCA to acquire and operate a mobile simulation lab to provide hands-on training for providers in rural communities. The Year 1 budget includes \$750,000 for customized vehicle; \$117,615 for high-fidelity manikin and \$40,000 for trauma limbs; \$140,060 for simulation software and 15 training modules; \$6,392 for virtual headsets; and \$90,000 for medical training supplies. All pricing is vendor-provided.

Staff for the mobile simulation lab include a 1.0 FTE registered nurse trainer with annual salary and benefits of \$118,750 and a 1.0 FTE program manager with annual salary and benefits of \$93,750, assuming these positions will be in place for four and six months in Year 1, respectively. These salaries are consistent with CCA's existing compensation structure.

Program 3: Recruitment and Retention Program. This program includes three projects.

B. Rural Nurse Residency. KDHE will sub-award CCA to lead a statewide rural nurse residency program for recent nursing school graduates. The Year 1 budget includes \$215,000 to license a nationally recognized curriculum for the program.

C. Rural Instructor and Preceptor Payments. The Year 1 budget includes \$500,000 for KDHE to sub-award CCA to make adjunct faculty and preceptor payments to expand nurse training opportunities in rural communities. (These payments are not payment for delivery of clinical

services, but to encourage clinicians to serve as instructors and preceptors.) CCA will contract with individual preceptors or with hospitals to disburse these payments to individuals.

D. Behavioral Health Apprenticeships. KDHE will sub-award KDADS to contract with Healthworks for this program. The Year 1 budget of \$65,460 will fund Healthworks to design and set-up pathways for behavioral health apprenticeships, which will then be opened for participation in Year 2. If a student's participation in such apprenticeship qualifies the student for a credential necessary to secure employment in a healthcare role, the participating student will be subject to a five-year rural practice commitment.

Program 4: Career Exploration Program. This program includes four projects, each to be executed through KDHE's sub-award to CCA.

A. Rural Chapters of HOSA – Future Health Professions. The State has budgeted for CCA to sub-award the University of Kansas Medical Center Institute for Community Engagement to expand rural chapters of the student organization HOSA – Future Health Professions. The Year 1 budget includes staffing, supplies, and indirect costs.

B. K-12 Health Science Career Technical Education. The State has budgeted \$216,000 in Year 1 for CCA to contract with and expand these programs in 10 rural schools (with more schools added in subsequent years.) The estimated cost per school is \$21,600 to include instructor stipends, student transportation, and training equipment.

C. High School Certification/Pre-Apprenticeships. Funding for this Project will begin in Year 2.

D. Regional Career Expo. The State has budgeted \$40,000 in Year 1 for CCA to plan, promote, and host regional career expos in addition to the successful annual statewide healthcare career expo. These regional events will provide students unable to travel to the annual event the opportunity to explore health care careers.

Budget Narrative – Initiative 4

Initiative 4 Programs	
1	Evidence-Based Practices Program
2	ACO Readiness Program
3	Transportation Program
4	Medicaid Provider Incentive Payment Program

The following table summarizes the SF424A expenditure plan for Initiative 4: *Enable Value-Based Care* for each of the five performance years.

TABLE 12 Initiative 4 Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel	\$12,751.96	\$0	\$0	\$0	\$0
B. Fringe	\$4,831.07	\$0	\$0	\$0	\$0
C. Travel	\$0	\$0	\$0	\$0	\$0
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$0	\$0	\$0	\$0	\$0
F. Contract	\$46,187,423.47	\$20,566,244	\$25,717,024	\$20,829,399	\$23,415,925
H. Other	\$0	\$31,750,000	\$33,300,000	\$38,400,000	\$40,600,000
I. Direct Charges	\$46,205,006.50	\$52,316,244	\$59,017,024	\$59,229,399	\$64,015,925
J. Indirect Charges	\$0	\$252,746	\$259,422	\$266,362	\$274,471
Total	\$46,205,006.50	\$52,568,990	\$59,276,446	\$59,495,761	\$64,290,396

For the ten-month Budget Period 1 (December 31, 2025, to October 30, 2026), the State has budgeted the following amounts by category:

TABLE 12.A Year 1 Personnel – Salary and Wage					
Salary and Wages	Annual Salary	Total FTE	Months	Total Year 1 Cost	Program
BCHS Trauma Section Director, Wendy O'Hare	\$74,312.00	0.25	5	\$7,740.83	3A
Sr. Deputy Director Division of Public Health, Derik Flerlage	\$120,267.00	0.1	5	\$5,011.13	3A

The State also has budgeted for fringe benefits for the personnel identified above:

TABLE 12.B Year 1 Personnel – Fringe Benefits				
Fringe Benefit	Rate	Salary Requested	Total Year 1 Cost	Program
BCHS Trauma Section Director Fringe	20.94%	\$7,740.83	\$1,620.93	3A
Sr. Deputy Director Division of Public Health Fringe	20.94%	\$5,011.13	\$1,049.33	3A
KDHE Personnel Insurance	\$14,817 x 0.35 FTE * 5 months		\$2,160.81	3A

The following tables detail by category the Year 1 expenses included in the contracts into which the State will enter to implement Initiative 4 programs (listed under “E. Contract” in Table 12).

TABLE 12.F.1 Year 1 Personnel Expenses Included in Contracts					
Position	Annual Salary and Benefits	FTEs	Months	Total Year 1 Cost	Program
Performance Improvement Specialist	\$151,125.00	1	6	\$75,562.50	1
Metrics Data Manager	\$134,759.00	1	6	\$67,379.50	1
Certified Coder	\$73,008.00	1	6	\$36,504.00	3D
Medicaid Practice Facilitator	\$131,250.00	1	6	\$65,625.00	4

TABLE 12.E.2 Year 1 Travel Expenses Included in Contracts			
Purpose	Miles @ .725/mile	Total Year 1 Cost	Program
Performance Visits to Rural Sites – 14 visits x 200 miles	2,800	\$2,030.00	1
Medicaid Clinic Site Visits	9,100	\$6,597.50	4

TABLE 12.E.4 Year 1 Third-Party Contracts (to include detailed breakdown with submitted contracts)		
Description	Total Year 1 Cost	Program
QHi Platform	\$232,055.00	1
Evidence-Based Practice Infrastructure Assistance Fund	\$22,000,000.00	1
Evidence-Based Practice Provider Incentive Payments	\$11,000,000.00	1
ACO Readiness Program	\$3,400,000.00	2
EMS Interfacility Transport Grant	\$870,958.97	3A
Behavioral Health Crisis Transport	\$2,460,000.00	3B
Non-Emergency 911 Calls	\$3,250,000.00	3C
EMS Treatment in Place/Alternate Site	\$1,480,000.00	3D
Medicaid Care Coordination Incentives	\$1,000,000.00	4

TABLE 12.J Year 1 Indirect Costs Included in Contracts		
Description	Total Year 1 Cost	Program
Behavioral Health Transport	\$246,229.00	3B

The following discussion details Initiative 4 expenditures by individual program.

Program 1: Evidence-Based Practices Program. All rural hospitals and all rural health clinics and private physician practices, FQHCs, and CCBHCs located in rural Kansas (collectively, “Clinics”) will be eligible to participate in this program upon execution of a Memorandum of Understanding. KDHE will sub-award CCA to administer a \$22M Infrastructure Assistance Fund to contract participating hospitals and Clinics to develop and maintain the capabilities and capacity needed to successfully implement and sustain evidence-based practices and collect and report data on specified performance measures. Such infrastructure may include but is not limited to dedicated clinical and administrative staff time, engagement with external performance improvement specialists, technology upgrades, mandatory medical and clinical staff training, and development of specified data collection and reporting capabilities.

Participating hospitals and Clinics will be eligible to receive payments (\$50,000 for rural hospitals, \$25,000 for Clinics) if they successfully submit complete data in a timely manner on specified performance measures demonstrating adherence to evidence-based practices for the third calendar quarter of 2026.

The Year 1 budget assumes all eligible rural hospitals and Clinics will participate in the program, receive support from the Infrastructure Assistance Fund (budgeted at \$100,000 per hospital and \$50,000 per Clinic), and qualify for pay-for-reporting payments.

Participating providers will utilize a well-established quality reporting platform with which rural providers are familiar, QHi, to support quarterly reporting across all five years of the program. CCA will contract with the Kansas Hospital Association, which maintains QHi, to make the platform available to all providers eligible for the program for data reporting purposes at a cost of \$232,055 in Year 1. This amount includes programming and technical support as well as annual subscription fees for rural health clinics, private physician practices, FQHCs, and CCBHCs (subscription fees for rural hospitals already are covered by the FLEX program).

To support the program, CCA will employ a performance improvement specialist to work directly with hospitals and Clinics in implementing and sustaining evidence-based guidelines with annual salary and benefits of \$151,125 (consistent with salaries of current CCA employees in similar roles). The budget includes \$2,030 in travel (mileage only) for performance improvement specialist site visits. CCA also will employ a metrics data manager for the same period to support hospitals and Clinics in compiling and submitting data, reviewing data as it is submitted, and making payments to providers. The annual salary and benefits for this position will be \$134,759 (consistent with salaries of current TUKHS employees in similar roles), assuming these positions will be in place for six months during Year 1.

Program 2: ACO Readiness Program. KDHE will sub-award CCA to contract with a successful national vendor under the Medicare Shared Savings Program and ACO REACH models, Belong Health, to operate a “shadow” ACO for rural Kansas providers that have not participated in these programs to date. Based on information provided by the vendor with which CCA presently contracts, the State has budgeted for an administrative fee of \$44.00 per attributed beneficiary per month (PBPN, with up to \$5 PBPN in quality performance incentives. In addition to direct practice engagement, participants will be eligible for "value-based activity incentives" of \$125, \$75, and \$50 for annual wellness visits, Inpatient timely follow-up within 14 days of discharge, and Emergency Department timely follow-up within 14 days of ED visit, mirroring best practice programs. The Year 1 budget for the ACO Readiness Program is \$3.4 million, with expanded participation expected in subsequent years.

Program 3 Transportation Program. This program includes five projects:

A. Interfacility Transport Teams. In Year 1, KDHE will conduct a competitive regional grant application (RFA) to sub-award an interfacility transport team. Cost estimates were based on equipment and personnel budgets provided by rural EMS providers.

KDHE will also allocate 25% of the KDHE Trauma Section Director's time to overseeing the Interfacility Transports project, as well as 10% of the Senior Deputy Director of the Division of Public Health. These positions have annual salaries, respectively, of \$74,312 and \$120,267 and will be in place for 5 months of Year 1. Fringe is budgeted at the same rate as Administrative KDHE roles (20.94% of salary request, \$14,817 x FTE request).

B. Behavioral Health Crisis Transports. KDHE has budgeted \$2.46 million in Year 1 to sub-award KDADS, who will contract with a to-be-determined vendor to establish and operate a statewide behavioral health crisis transport network. The vendor will be selected via a competitive procurement process. The budget was based on vendor-provided pricing to deliver 24/7 secure patient transportation en route to and from hospitals, treatment centers, and crisis centers across Kansas to facilitate admissions and treatment during crisis episodes, reducing impact on rural law enforcement and hospital emergency rooms.

C. Non-Emergency 911 Calls. KDHE will contract with Global Medical Response, an experienced vendor, to integrate technology with rural call centers to connect to a team of trained nurse navigators. Via their 911 REACT tool, the GMR team will guide patient transport to non-emergent resources, as appropriate. When hospital care is needed, the team will identify available beds, align with payer networks, secure authorizations, and dispatch transport. The Year 1 budget for this program is \$3.25M based on GMR's proposal.

D. EMS Treatment in Place/Transport to Alternative Location. KDHE has budgeted \$1.48 million to sub-award CCA to reimburse claims submitted by rural EMS providers for (1) rendering in-home or on-site care for patients not requiring hospital-level care and (2) transporting patients to an alternative location if a clinic or other location is more appropriate than a hospital for the patient's needs. These services are not fully reimbursed by insurance, and RHTP funds will only be used for services and individuals who would not be eligible for insurance reimbursement. The budget includes a certified coder to be employed by CCA at an annual salary and benefits of \$73,008 to process claims, assuming this position will be in place for six months in Year 1.

E. Non-Emergency Transportation Grant Program. KDHE will sub-award local communities to implement innovative and sustainable local solutions to address the lack of local transportation for medical services. This program will officially launch in Year 2.

Program 4: Medicaid Provider Incentive Payment Program. To address the difficulties the State's Medicaid managed care organizations report in engaging rural beneficiaries to care coordination services, KDHE will sub-award CCA to contract with and offer an incentive payment of \$1,000 for rural primary care providers who successfully connect Medicaid beneficiaries to

MCO care coordination services. The State has budgeted \$1 million for these incentive payments for Year 1.

CCA will provide a practice facilitator who will assist rural practice incorporate patient identification and consent into clinic workflows at an annual rate of \$131,250, assuming this position will be in place for six months in Year 1), consistent with CCA's current compensation rates for similar positions. The budget also includes \$6,598 in travel expenses (mileage only) for practice facilitator site visits to rural clinics.

Budget Narrative – Initiative 5

Initiative 5 Programs	
1	Remote Patient Monitoring Program
2	Telehealth Navigator Program
3	Data Infrastructure Program
4	Emerging Technology Program

The following table summarizes the SF424A expenditure plan for Initiative 5: *Harness Data and Technology* for each of the five performance years:

TABLE 13 Initiative 5 Expenditure Plan					
Category	Year 1	Year 2	Year 3	Year 4	Year 5
A. Personnel/Fringe	\$0	\$0	\$0	\$0	\$0
C. Travel	\$0	\$0	\$0	\$0	\$0
D. Equipment	\$0	\$0	\$0	\$0	\$0
E. Supplies	\$0	\$0	\$0	\$0	\$0
F. Contract	\$32,175,369.40	\$15,974,916	\$17,634,346	\$22,399,916	\$24,791,330
H. Other	\$0	\$24,694,000	\$15,694,000	\$24,694,000	\$730,000
I. Direct Charges	\$32,175,369.40	\$40,668,916	\$33,328,346	\$47,093,916	\$25,521,330
J. Indirect Charges	\$0	\$138,000	\$138,000	\$138,000	\$138,000
Total	\$32,175,369.40	\$40,806,916	\$33,466,346	\$47,231,916	\$25,659,330

The following tables detail by category the Year 1 expenses included in the contracts into which the State will enter to implement Initiative 5 programs (listed under “E. Contract” in Table 13).

TABLE 13.E.1 Year 1 Personnel Expenses Included in Contracts					
Position	Annual Salary and Benefits	FTEs	Months	Total Year 1 Cost	Program
Telehealth RN Care Coordinators	\$109,200.00	3.0	6	\$163,800.00	2
CCBHC Data Scientist	\$147,000.00	.25	6	\$18,375.00	3D

TABLE 13.E.2 Year 1 Travel Expenses Included in Contracts			
Purpose	Miles @ .725/mile	Total Year 1 Cost	Program
Telehealth Coordinators 3 x 8 sites x 250 miles	6,000	\$4,350.00	2

TABLE 13.E.3 Year 1 Equipment Expenses Included in Contracts				
Description	Price per unit	No.	Total Year 1 Cost	Program
Data Platform	\$180,000.00	1	\$180,000.00	3D

TABLE 13.E.4 Year 1 Supply Expenses Included in Contracts				
Description	Cost	Number	Total Year 1 Cost	Program
Technology and Supply Bundle	\$2,330.00	3	\$6,990.00	2

TABLE 13.E.5 Year 1 Third-Party Contracts (to include detailed breakdown with submitted contracts)		
Description	Total Year 1 Cost	Program
BioIntelliSense Remote Inpatient/Post-acute Monitoring at anchor hospitals	\$7,014,500.00	1
BioIntelliSense Remote Monitoring at non-anchor hospitals	\$2,500,000.00	1
AHAP project management services	\$150,000.00	1
Health Data Utility landscape assessment and data strategy	\$987,500.00	3A
Diabetes Data Dock	\$554,217.00	3C
Oracle Clinical AI Agent	\$5,964,000.00	4
Technology Grants	\$13,692,461.21	4
BioSTL Consulting Services	\$750,000.00	4
Kansas Health Institute Facilitation Services	\$79,692.96	4

TABLE 13.J Year 1 Indirect Expenses Included in Contract		
Description	Total Year 1 Cost	Program
Diabetes Data Dock	\$55,422.00	3C

The following discussion details Initiative 5 expenditures by individual program.

Program 1: Remote Monitoring. KDHE will sub-award CCA to contract with BioIntelliSense to implement with AHAP's support a centralized continuous remote patient monitoring program for inpatient and post-discharge patients beginning with ten rural anchor hospitals in Year 1 (approximately 350 medical/surgical beds and up to 450 post-discharge patients). The State selected BioIntelliSense for this program based on the unique capabilities of its FDA-cleared wearable devices, its experience and success designing and implementing centralized monitoring programs across multiple facilities, its commitment to providing ongoing support for monitoring programs, and the results health systems have achieved implementing BioIntelliSense's program, including lower length of stay, reduced complication rates, higher patient satisfaction scores, and reduced workloads for hospital staff.

The Year 1 budget is structured as a fixed-cost program to mitigate risk and ensure cost predictability, with BioIntelliSense underwriting supply-chain and deployment risks. The Year 1 budget includes \$3.7M in up-front investments to establish the program and \$3.3M in annual/ongoing costs:

TABLE 14.1 Year 1 RPM at AHAP Participating Hospitals <i>Breakdown of Remote Inpatient/Post-acute Monitoring line item in Table 13.E.5</i>		
Line Item	Amount	Explanation
A. One-time program design and implementation costs	\$1,140,000.00	Infrastructure, integration, training, and program launch
B. One-time EMR training and participating hospital setup fees	\$2,000,000.00	One-time IT, EMR, cybersecurity, and application integration, training, setup, and support (~\$200k per facility across 10 sites)
C. One-time capital expenditures	\$525,000.00	One-time capital expenses (Gateways, Charging Stations, CapEx per Bed, Install and Setup Training) for all Year 1 Participating Hospitals
D. Annual analytics and reporting for program	\$272,000.00	Facility level and statewide RPM Reporting and Analytics Service
E. Annual Command Center Centralized Participating Hospital License	\$125,000.00	Annual license for the RHTP-funded centralized command center.

F. Annual Command Center Centralized Participating Hospital and Post-Acute Monitoring Setup, Training, and Labor	\$1,225,000.00	24/7 centralized clinical monitoring for Participating Hospital and post-acute patients. The centralized clinical monitor will provide services according to the service requirements outline for Center Centralized Inpatient and Post-Acute Monitoring Exhibit 5. The centralized clinical monitor's contract will require services under its Year 1 contract until the last activated Bio Button and/or Bio Button Pro annual license expires ² . Contractor will negotiate terms and contract with a service provider acceptable to CCA and AHAP; Contractor will pay centralize clinical monitor's fee.
G. Annual SaaS Bed Licensing	\$875,000.00	Per- Bio Button / Pro subscription for ~350 beds and patches (\$2,050 per bed per year).
H. Wearable devices & adhesives supply guarantee (fixed-cost provision) via a "Bill & Hold" model.	\$717,500.00	Fixed, all-inclusive supply for devices and patches; vendor underwrites utilization and supply-chain variability.
I. Post-acute monitoring kits & services	\$135,000.00	At-home kits and data service at \$300 per post-acute patient (inclusive of Bio Button / Pro home kit, BioHub Cellular gateway, up to 3 months data service) for 450 patients in Year 1.

The Year 1 budget also includes \$2.5M for CCA to contract with BioIntelliSense to implement and maintain remote monitoring services at non-anchor rural Kansas hospitals, identified by CCA and approved by KDHE. The costs will be of similar structure to those for anchor hospitals.

Finally, the Year 1 budget includes \$150k for CCA to contract with AHAP, LLC to oversee and support the implementation of BioIntelliSense at anchor hospitals.

Program 2: Telehealth Navigator Program. KDHE will sub-award CCA to operate the program with three registered nurse care coordinators as telehealth navigators at an annual salary and benefit rate of \$109,200, assuming these positions will be in place for six months in Year 1. The budget includes the standard technology and supply bundle for the three telehealth navigators and mileage reimbursement of \$4,350 for navigators' travel to rural clinics to facilitate telehealth visits and follow-up services.

² For avoidance of doubt and example, if the last Bio Button or Bio Button Pro is activated on August 30, 2027, the centralized clinical monitor will provide required monitoring until August 29, 2028 without any additional payment. If a Bio Button Pro replaces a Bio Button on September 15, 2027, the monitoring will still end on August 29, 2028.

Program 3: Data Infrastructure Program. This program includes five projects.

A. Kansas Health Data Utility and All-Payer Claims Database (APCD). KDHE will contract with a to-be-determined entity to provide a comprehensive landscape analysis and data strategy, at no more than \$1M in Year 1. The vendor will create a strategic plan to establish a Health Data Utility and eventually an All-Payer Claims Database in Kansas, but the exact timeline will depend on the initial assessment.

B. HIE Connectivity for Rural Providers. This program will commence in Year 2; no funds are budgeted for Year 1.

C. Data Dock – Diabetes Risk Monitoring and Intervention. The State has budgeted \$609,639.00 in Year 1 to sub-award CCA to sub-award the University of Kansas Medical Center Department of Endocrinology, Diabetes, and Clinical Pharmacology to deploy Data Dock, a platform to synthesize EHR and continuous glucose monitoring device data that enables rural providers to detect and respond to patient deterioration. Year 1 includes deployment at three rural pilot sites. Budgeted expenses include personnel (none of which constitutes payment for provider patient care services; instead, they relate to program development, operations, and oversight), contracted consultant services, software subscriptions, cloud computing costs, IT integration with pilot sites, and indirect costs.

D. CCBHC Data Center. KDHE will sub-award KDADS to contract the Association of Community Mental Health Centers to establish a data warehouse to meet CCBHC reporting requirements and continuous quality improvement needs. The Year 1 budget includes \$180,000 for platform development and \$36,750 for a 0.25 FTE data scientist contracted position.

E. Kansas Data Trust. See project A.

Program 4: Emerging Technology. KDHE has budgeted \$5,964,000 to sub-award CCA to contract with and provide those Kansas rural clinics utilizing the Oracle Community Works (approximately one-half of those clinics statewide) with access to Oracle's Clinical AI Agent. Based on a rate of \$1.50 per note, Year 1 cost is estimated at \$7,800 per rural provider.

To assist providers utilizing other EHR platforms in identifying similar technologies, CCA will convene an Emerging Technology Committee with subject matter experts to evaluate and recommend technology solutions that meet the specific needs of rural providers. Through its subaward with KDHE, CCA will contract with BioSTL to serve as the subject matter expert to

support the Technology Committee. This panel will help rural providers select the artificial intelligence solutions and other technologies most appropriate for their circumstances.

BioSTL is a St. Louis–based nonprofit organization that connects healthcare organizations with cutting-edge technology solutions. Since 2001, BioSTL has developed nationally recognized programs that help healthcare systems identify, evaluate, and adopt high-value technologies. BioSTL regularly curates and convenes expert committees to assess promising health technologies, ensuring they align with provider needs and deliver measurable impact. The State has budgeted \$600,000 in Year 1 for BioSTL’s consulting services based on the firm’s estimated time and effort for this engagement. The State also has budgeted \$150,000 for registration fees to permit information technology professionals working for Kansas rural providers to participate in BioSTL’s quarterly Innovation Sharing events and annual Innovation Summit.

CCA also will contract with the Kansas Health Institute (KHI) to facilitate the Technology Committee at a rate of \$79,692.96 in Year 1 based on KHI’s estimated time and effort for this engagement. Established in 1995, KHI is a nonprofit, nonpartisan organization that provides evidence-based research, policy analysis, and educational support to improve the health of Kansans. KHI brings deep experience convening stakeholders, guiding evidence-based deliberation, and ensuring that decisions are grounded in rigorous, unbiased analysis. Their longstanding role as a trusted resource for Kansas policymakers positions them well to support rural healthcare providers in evaluating and adopting innovations. KHI served as the facilitator for the Kansas Rural Health Innovation Alliance, the statewide stakeholder group that participated in the development of the State’s RHT Plan.

The State has also budgeted ~\$14M in Year 1 for an Emerging Technologies Grant Program. This program will not be limited to a certain type of artificial intelligence; Eligible Providers (as defined above in reference to the Regional Partnership Grant Program) can seek funds for any emerging technology with demonstrated capabilities to improve care delivery. KDHE will sub-award these Eligible Providers, selected via statewide competitive grant program.